NovoPsych

Obsessional Compulsive Inventory-Revised-Parent (ChOCI-R-P)

Instructions:

1 Each of the following questions asks you about things or "habits" you feel your son/daughter has to do although (s)he may know that they do not make sense. Sometimes, (s)he may try to stop from doing them but this might not be possible. (S)he might feel worried or angry or frustrated until (s)he has finished what (s)he has to do. An example of a habit like this may be the need to wash his/her hands over and over again even though they are not really dirty, or the need to count up to a special number (e.g. 6 or 10) while (s)he does certain things.

Please answer each question by choosing how much you agree with the statement, or how much you think it is true of your son/daughter. Please answer each item, without spending too much time on any one item. There are no right or wrong answers.

- (S)he spends far too much time washing his/her hands over and over again.

0 Not at all
1 Somewhat
2 A lot

		Not at all	Somewhat	A lot
2	(S)he feels (s)he must do ordinary/everyday things exactly the same way, every time (s)he does them.	(0)	1	2
3	(S)he spends a lot of time every day checking things over and over and over again.	(0)	1	2
4	(S)he often have trouble finishing things because (S)he needs to make absolutely sure that everything is exactly right.	(0)	1	2
5	(S)he spends far too much time arranging his/her things in order.	(0)	1	2
6	(S)he needs someone to tell him/her things are alright over and over again.	(0)	1	2
7	If (s)he touches something with one hand, (s)he feels (s)he absolutely must touch the same thing with the other hand, in order to make things even and equal.	(0)	1	2
8	(S)he always counts, even when doing ordinary things.	(0)	1	2
9	If (s)he has a 'bad thought', (s)he always has to make sure that (s)he immediately has a 'good thought' to	(0)	1	2
10	(S)he is often very late because (s)he keeps on repeating the same action, over and over again.	(0)	1	2

11 Please try to think about the three most upsetting HABITS that (s)he feels (s)he has to do and can't stop. For example, feeling that (s)he has to wash his/her hands far too often, or repeating the same action over & over, or constantly checking that the doors and windows are shut properly.

- Habit 1. She likes things in groups of 3.

~~She lines items up on her desk in a certain order when~~
~~She cleans~~ new
~~Can't think of a third~~ new

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12	Habit 2.
	She lines things up on her desk a certain way when she cleans.
13	Habit 3.
	Can't think of one.
14	How much time does (s)he spend doing these habits?
	0 None
	(1) Less than 1 hr. a day (occasionally)
	2 1-3 hrs. a day (part of a morning or afternoon)
	3 3-8 hrs. a day (about half the time you're awake)
	4 More than 8 hrs. a day (almost all the time you're awake)
15	How much do these habits get in the way of school or doing things with friends?
	(0) Not at all
	1 A little
	2 Somewhat
	3 A Lot
	4 Almost always
16	How would (s)he feel if prevented from carrying out their habits? How upset would they become?
	(0) Not at all
	1 A little
	2 Somewhat
	3 A lot
	4 Totally
17	How much does (s)he try to fight the upsetting habits?
	0 (S)he always tries to resist
	1 (S)he tries to resist most of the time
	2 (S)he makes some effort to resist
	3 Even though (s)he wants to, (s)he doesn't try to resist
	(4) He/she doesn't resist at all
18	How strong is the feeling that (s)he has to carry out the habits?
	0 Not strong
	(1) Mild pressure to carry out habits
	2 Strong pressure to carry out habits; hard to control
	3 Very strong pressure to carry out habits; very hard to control
	4 Extreme pressure to carry out habits; impossible to control

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19 How much has (s)he been avoiding doing anything, going any place, or being with anyone because of his/her upsetting habits?

(0) Not at all
1 A little
2 Somewhat
3 A lot
4 Almost always

20 In this section, each of the questions asks you about thoughts, ideas, or pictures that keep coming into the mind of your son or daughter, even though (s)he does not want them to do so. They may be unpleasant, silly, or embarrassing. For example, some young people have the repeated thought that germs or dirt are harming them or other people, or that something unpleasant may happen to them or someone special to them. These are thoughts that keep coming back, over and over again, even though the person does not want them.

Please answer each question by indicating which option best describes how much you agree with the statement, or how much you think it is true of your son/daughter. Please answer each item, without spending too much time on any one item. There are no right or wrong answers.

- (S)he can't stop thinking upsetting thoughts about an accident.

0 Not at all
1 Somewhat
2 A lot

21	(S)he often has bad thoughts that make him/her feel like a terrible person.	(0)	1	2
22	Upsetting thoughts about his/her family being hurt go round and round in his/her head and stop him/her from concentrating.	(0)	1	2
23	(S)he always has big doubts about whether (s)he made the right decision, even about stupid little things	0	(1)	2
24	(S)he can't stop upsetting thoughts about death from going round in his/her head, over and over again.	(0)	1	2
25	(S)he often has mean thoughts about other people that (s)he feels are terrible, over and over again.	(0)	1	2
26	(S)he often have horrible thoughts about going crazy.	(0)	1	2
27	(S)he keeps on having frightening thoughts that something terrible is going to happen and it will be his/her fault.	(0)	1	2
28	(S)he is very frightened that (s)he will think something (or do something) that will upset God	(0)	1	2
29	(S)he is always worried that her mean thoughts about other people are as wicked as actually doing mean things to them	(0)	1	2

30 Please list the three most severe THOUGHTS that (s)he often has and can't stop thinking about. For example, thinking about hurting someone, or thinking bad things about God.
- Thought 1.

None that she has ever told me

31 Thought 2.

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32	Thought 3.
33	How much time does (s)he spend thinking about these things?
	0 None
	1 Less than 1 hr. a day (occasionally)
	2 1-3 hrs. a day (part of a morning or afternoon)
	3 3-8 hrs. a day (about half the time you're awake)
	4 More than 8 hrs. a day (almost all the time you're awake)
34	How much do these thoughts get in the way of school or doing things with friends?
	0 Not at all
	1 A little
	2 Somewhat
	3 A Lot
	4 Extreme
35	How much do these thoughts bother or upset him/her?
	0 Not at all
	1 A little
	2 Somewhat
	3 A Lot
	4 Extreme
36	How hard does (s)he try to stop the thoughts or ignore them?
	0 (S)he always tries to resist
	1 (S)he tries to resist most of the time
	2 (S)he makes some effort to resist
	3 Even though (s)he wants to, (s)he doesn't try to resist
	4 (S)he doesn't resist at all
37	When (s)he tries to fight the thoughts, can (s)he beat them? How much control does (s)he have over the thoughts?
	0 Complete control
	1 Much control
	2 Moderate control
	3 Little control
	4 No control

38	How much has (s)he been avoiding doing anything, going any place, or being with anyone because of his/her thoughts?	
	0	Not at all
	1	A little
	2	Somewhat
	3	A Lot
	4	Almost always

Developer Reference:

Uher, R., Heyman, I., Turner, C. M., & Shafran, R. (2008). Self-, parent-report and interview measures of obsessive–compulsive disorder in children and adolescents. Journal of Anxiety Disorders, 22(6), 979-990. doi:10.1016/j.janxdis.2007.10.001

Child

Oxford CBT

Children's Obsessional Compulsive Inventory-Revised-Self Report (ChOCI-R-S)

OXFORD CBT

Patient Name Zoe Bitner

Today's Date 11/3/26

Each of the following questions asks you about things or "habits" you feel you have to do although you may know that they do not make sense. Sometimes, you may try to stop from doing them but this might not be possible. You might feel worried or angry or frustrated until you have finished what you have to do. An example of a habit like this may be the need to wash your hands over and over again even though they are not really dirty, or the need to count up to a special number (e.g. 6 or 10) while you do certain things.

Please answer each question by indicating how much you agree with the statement, or how much you think it is true of you. Please answer each item, without spending too much time on any one item. There are no right or wrong answers.

Example: I spend far too much time washing my hands over and over again. **0** Not at all **1** Somewhat **2** A lot

		Not at all	Somewhat	A lot
1	I feel I must do ordinary/everyday things exactly the same way, every time I do them.	○	●	○
2	I spend a lot of time every day checking things over and over and over again.	○	●	○
3	I often have trouble finishing things because I need to make absolutely sure that everything is exactly right.	●	○	○
4	I spend far too much time arranging my things in order.	○	●	○
5	I need someone to tell me things are alright over and over again.	●	○	○
6	If I touch something with one hand, I feel I absolutely must touch the same thing with the other hand, in order to make things even and equal.	○	●	○
7	I always count, even when doing ordinary things.	○	●	○
8	If I have "bad thought", I always have to make sure that I immediately have a "good thought" to cancel it out.	●	○	○
9	I am often very late because I keep on repeating the same action, over and over again.	●	○	○
10	Please try to think about the most upsetting habits that you feel you have to do and can't stop. For example, feeling that you have to wash your hands far too often, or repeating the same action over and over, or constantly checking that the doors and windows are shut properly.			
11	Habit 1 Cleaning under my nails			
12	Habit 2 Organizeing things to much			

13 Habit 3

Picking at my scalp

14 How much time do you spend doing these habits?

- 0 None
- 1 Less than 1 hour a day (occasionally)
- 2 1-3 hours a day (part of a morning or afternoon)
- 3 3-8 hours a day (about half the time you're awake)
- 4 More than 8 hours a day (almost all the time you're awake)

15 How much do these habits get in the way of school or doing things with friends?

- 0 Not at all
- 1 A little
- 2 Somewhat
- 3 A lot
- 4 Almost always

16 How would you feel if prevented from carrying out your habits? How upset would you become?

- 0 Not at all
- 1 A little
- 2 Somewhat
- 3 A lot
- 4 Almost always

17 How much do you try to fight the upsetting habits?

- 0 I always try to resist
- 1 I try to resist most of the time
- 2 I make some effort to resist
- 3 Even though I want to, I don't try to resist
- 4 I don't resist at all

18 How strong is the feeling that you have to carry out the habits?

- 0 Not strong
- 1 Mild pressure to carry out habits
- 2 Strong pressure to carry out habits, hard to control
- 3 Very strong pressure to carry out habits, very hard to control
- 4 Extreme pressure to carry out habits, impossible to control

19 How much have you been avoiding doing anything, going any place or being with anyone because of your upsetting habits?

- 0 Not at all
- 1 A little
- 2 Somewhat
- 3 A lot
- 4 Almost always

Part Two: In this section, each of the questions asks you about "thoughts", ideas or pictures that keep coming into your mind even though you do not want them to do so. They may be unpleasant, silly, or embarrassing. For example, some young people have the repeated thought that germs or dirt are harming them or other people, or that something unpleasant may happen to them or someone special to them. These are thoughts that keep coming back, over and over again, even though you do not want them.

Please answer each question by choosing how much you agree with the statement, or how much you think it is true of you. Please answer each item, without spending too much time on any one item. There are no right or wrong answers.

Example: I can't stop thinking upsetting thoughts about an accident. **0** Not at all **1** Somewhat **2** A lot

		Not at all	Somewhat	A lot
21	I often have bad thoughts that make me feel like a terrible person.	●	○	○
22	Upsetting thoughts about my family being hurt go round and round in my head and stop me from concentrating.	○	●	○
23	I always have big doubts about whether I've make the right decision, even about stupid little things.	○	●	○
24	I can't stop upsetting thoughts about death from going round in my head, over and over again.	●	○	○
25	I often have mean thoughts about other people that I feel are terrible, over and over again.	●	○	○
26	I often have horrible thoughts about going crazy.	●	○	○
27	I keep on having frightening thoughts that something terrible is going to happen and it will be my fault.	●	○	○
28	I'm very frightened that I will think something (or do something) that will upset God.	●	○	○
29	I'm always worried that my mean thoughts about other people are as wicked as actually doing mean things to	●	○	○

30 Please list the three most severe thoughts that you often have and can't stop thinking about. For example, thinking about hurting someone, or thinking bad things about God.

none

31 Thought 1 none

32 Thought 2 none

33 Thought 3 none

34 How much time do you spend thinking about these things?

- 0 None
- 1 Less than 1 hour a day (occasionally)
- 2 1-3 hours a day (part of a morning or afternoon)
- 3 3-8 hours a day (about half the time you're awake)
- 4 More than 8 hours a day (almost all the time you're awake)

35 How much do these thoughts bother or upset you?

- 0 Not at all
- 1 A little
- 2 Somewhat
- 3 A lot
- 4 Extreme

36 How hard do you try to stop the thoughts or ignore them?

- 0 I always try to resist
- 1 I try to resist most of the time
- 2 I make some effort to resist
- 3 Even though I want to, I don't try to resist
- 4 I don't resist at all

37 When you try to fight the thoughts, can you beat them? How much control do you have over the thoughts?

- 0 Complete control
- 1 Much control
- 2 Moderate control
- 3 Little control
- 4 No control

38 How much have you been avoiding doing anything, going any place, or being with anyone because of your thoughts?

- 0 Not at all
- 1 A little
- 2 Somewhat
- 3 A lot
- 4 Almost always

Developer Reference:

Uher, R., Heyman, I., Turner, C. M., & Shafran, R. (2008). Self-parent-report and interview measures of obsessive–compulsive disorder in children and adolescents. Journal of Anxiety Disorders, 22(6), 979-990. doi:10.1016/j.janxdis.2007.10.001

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John N. Constantino, MD

Assessment ID _____

SRS™-2 AutoScore™ Form — School-Age — ○ MALE ○ FEMALE

INSTRUCTIONS

For each question, please darken the circle that best describes this child's behavior **over the past 6 months.**

Child's name Zoe Bittner — Child's age in years _____

Rater's name Mrs. Weber — Date of rating _____

Relationship to rated individual ☐ Mother ☐ Father ☐ Other custodial adult ☐ Teacher ☐ Other specialist

Grade _____ School or clinic _____

PLEASE PRESS HARD WHEN MARKING YOUR RESPONSES.

1 = NOT TRUE 2 = SOMETIMES TRUE 3 = OFTEN TRUE 4 = ALMOST ALWAYS TRUE

1.	Seems much more fidgety in social situations than when alone.	1	2	3	4
2.	Expressions on his or her face don't match what he or she is saying.	1	2	3	4
3.	Seems self-confident when interacting with others.	1	2	3	4
4.	When under stress, he or she shows rigid or inflexible patterns of behavior that seem odd.	1	2	3	4
5.	Doesn't recognize when others are trying to take advantage of him or her.	1	2	3	4
6.	Would rather be alone than with others.	1	2	3	4
7.	Is aware of what others are thinking or feeling.	1	2	3	4
8.	Behaves in ways that seem strange or bizarre.	1	2	3	4
9.	Clings to adults, seems too dependent on them.	1	2	3	4
10.	Takes things too literally and doesn't get the real meaning of a conversation.	1	2	3	4
11.	Has good self-confidence.	1	2	3	4
12.	Is able to communicate his or her feelings to others.	1	2	3	4
13.	Is awkward in turn-taking interactions with peers (for example, doesn't seem to understand the give-and-take of conversations).	1	2	3	4
14.	Is not well coordinated.	1	2	3	4
15.	Is able to understand the meaning of other people's tone of voice and facial expressions.	1	2	3	4
16.	Avoids eye contact or has unusual eye contact.	1	2	3	4
17.	Recognizes when something is unfair.	1	2	3	4
18.	Has difficulty making friends, even when trying his or her best.	1	2	3	4
19.	Gets frustrated trying to get ideas across in conversations.	1	2	3	4
20.	Shows unusual sensory interests (for example, mouthing or spinning objects) or strange ways of playing with toys.	1	2	3	4
21.	Is able to imitate others' actions.	1	2	3	4
22.	Plays appropriately with children his or her age.	1	2	3	4
23.	Does not join group activities unless told to do so.	1	2	3	4
24.	Has more difficulty than other children with changes in his or her routine.	1	2	3	4
25.	Doesn't seem to mind being out of step with or "not on the same wavelength" as others.	1	2	3	4
26.	Offers comfort to others when they are sad.	1	2	3	4
27.	Avoids starting social interactions with peers or adults.	1	2	3	4
28.	Thinks or talks about the same thing over and over.	1	2	3	4
29.	Is regarded by other children as odd or weird.	1	2	3	4
30.	Becomes upset in a situation with lots of things going on.	1	2	3	4
31.	Can't get his or her mind off something once he or she starts thinking about it.	1	2	3	4
32.	Has good personal hygiene.	1	2	3	4

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PLEASE PRESS HARD WHEN MARKING YOUR RESPONSES.

1 = NOT TRUE 2 = SOMETIMES TRUE 3 = OFTEN TRUE 4 = ALMOST ALWAYS TRUE

33. Is socially awkward, even when he or she is trying to be polite. 1 2 3 4
34. Avoids people who want to be emotionally close to him or her. 1 2 3 4
35. Has trouble keeping up with the flow of a normal conversation. 1 2 3 4
36. Has difficulty relating to adults. 1 2 3 4
37. Has difficulty relating to peers. 1 2 3 4
38. Responds appropriately to mood changes in others (for example, when a friend's or playmate's mood changes from happy to sad). 1 2 3 4
39. Has an unusually narrow range of interests. 1 2 3 4
40. Is imaginative, good at pretending (without losing touch with reality). 1 2 3 4
41. Wanders aimlessly from one activity to another. 1 2 3 4
42. Seems overly sensitive to sounds, textures, or smells. 1 2 3 4
43. Separates easily from caregivers. 1 2 3 4
44. Doesn't understand how events relate to one another (cause and effect) the way other children his or her age do. 1 2 3 4
45. Focuses his or her attention to where others are looking or listening. 1 2 3 4
46. Has overly serious facial expressions. 1 2 3 4
47. Is too silly or laughs inappropriately. 1 2 3 4
48. Has a sense of humor, understands jokes. 1 2 3 4
49. Does extremely well at a few tasks, but does not do as well at most other tasks. 1 2 3 4
50. Has repetitive, odd behaviors such as hand flapping or rocking. 1 2 3 4
51. Has difficulty answering questions directly and ends up talking around the subject. 1 2 3 4
52. Knows when he or she is talking too loud or making too much noise. 1 2 3 4
53. Talks to people with an unusual tone of voice (for example, talks like a robot or like he or she is giving a lecture). 1 2 3 4
54. Seems to react to people as if they are objects. 1 2 3 4
55. Knows when he or she is too close to someone or is invading someone's space. 1 2 3 4
56. Walks in between two people who are talking. 1 2 3 4
57. Gets teased a lot. 1 2 3 4
58. Concentrates too much on parts of things rather than seeing the whole picture. For example, if asked to describe what happened in a story, he or she may talk only about the kind of clothes the characters were wearing. 1 2 3 4
59. Is overly suspicious. 1 2 3 4
60. Is emotionally distant, doesn't show his or her feelings. 1 2 3 4
61. Is inflexible, has a hard time changing his or her mind. 1 2 3 4
62. Gives unusual or illogical reasons for doing things. 1 2 3 4
63. Touches others in an unusual way (for example, he or she may touch someone just to make contact and then walk away without saying anything). 1 2 3 4
64. Is too tense in social settings. 1 2 3 4
65. Stares or gazes off into space. 1 2 3 4

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John N. Constantino, MD

Assessment ID

SRS™-2 AutoScore™ Form — School-Age

○ MALE ⊘ FEMALE

INSTRUCTIONS

For each question, please darken the circle that best describes this child's behavior **over the past 6 months.**

Child's name Zoe Bitner

Child's age in years 12

Rater's name Mr. Daly

Date of rating 10/26/26

Relationship to rated individual ☐ Mother ☐ Father ☐ Other custodial adult ☑ Teacher ☐ Other specialist

Grade 6 School or clinic SMES

PLEASE PRESS HARD WHEN MARKING YOUR RESPONSES.

1 = NOT TRUE 2 = SOMETIMES TRUE 3 = OFTEN TRUE 4 = ALMOST ALWAYS TRUE

1.	Seems much more fidgety in social situations than when alone.	1	2	3	4
2.	Expressions on his or her face don't match what he or she is saying.	1	2	3	4
3.	Seems self-confident when interacting with others.	1	2	3	4
4.	When under stress, he or she shows rigid or inflexible patterns of behavior that seem odd.	1	2	3	4
5.	Doesn't recognize when others are trying to take advantage of him or her.	1	2	3	4
6.	Would rather be alone than with others.	1	2	3	4
7.	Is aware of what others are thinking or feeling.	1	2	3	4
8.	Behaves in ways that seem strange or bizarre.	1	2	3	4
9.	Clings to adults, seems too dependent on them.	1	2	3	4
10.	Takes things too literally and doesn't get the real meaning of a conversation.	1	2	3	4
11.	Has good self-confidence.	1	2	3	4
12.	Is able to communicate his or her feelings to others.	1	2	3	4
13.	Is awkward in turn-taking interactions with peers (for example, doesn't seem to understand the give-and-take of conversations).	1	2	3	4
14.	Is not well coordinated.	1	2	3	4
15.	Is able to understand the meaning of other people's tone of voice and facial expressions.	1	2	3	4
16.	Avoids eye contact or has unusual eye contact.	1	2	3	4
17.	Recognizes when something is unfair.	1	2	3	4
18.	Has difficulty making friends, even when trying his or her best.	1	2	3	4
19.	Gets frustrated trying to get ideas across in conversations.	1	2	3	4
20.	Shows unusual sensory interests (for example, mouthing or spinning objects) or strange ways of playing with toys.	1	2	3	4
21.	Is able to imitate others' actions.	1	2	3	4
22.	Plays appropriately with children his or her age.	1	2	3	4
23.	Does not join group activities unless told to do so.	1	2	3	4
24.	Has more difficulty than other children with changes in his or her routine.	1	2	3	4
25.	Doesn't seem to mind being out of step with or "not on the same wavelength" as others.	1	2	3	4
26.	Offers comfort to others when they are sad.	1	2	3	4
27.	Avoids starting social interactions with peers or adults.	1	2	3	4
28.	Thinks or talks about the same thing over and over.	1	2	3	4
29.	Is regarded by other children as odd or weird.	1	2	3	4
30.	Becomes upset in a situation with lots of things going on.	1	2	3	4
31.	Can't get his or her mind off something once he or she starts thinking about it.	1	2	3	4
32.	Has good personal hygiene.	1	2	3	4

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PLEASE PRESS HARD WHEN MARKING YOUR RESPONSES.

1 = NOT TRUE 2 = SOMETIMES TRUE 3 = OFTEN TRUE 4 = ALMOST ALWAYS TRUE

33. Is socially awkward, even when he or she is trying to be polite. 1 2 3 4
34. Avoids people who want to be emotionally close to him or her. 1 2 3 4
35. Has trouble keeping up with the flow of a normal conversation. 1 2 3 4
36. Has difficulty relating to adults. 1 2 3 4
37. Has difficulty relating to peers. 1 2 3 4
38. Responds appropriately to mood changes in others (for example, when a friend's or playmate's mood changes from happy to sad). 1 2 3 4
39. Has an unusually narrow range of interests. 1 2 3 4
40. Is imaginative, good at pretending (without losing touch with reality). 1 2 3 4
41. Wanders aimlessly from one activity to another. 1 2 3 4
42. Seems overly sensitive to sounds, textures, or smells. 1 2 3 4
43. Separates easily from caregivers. 1 2 3 4
44. Doesn't understand how events relate to one another (cause and effect) the way other children his or her age do. 1 2 3 4
45. Focuses his or her attention to where others are looking or listening. 1 2 3 4
46. Has overly serious facial expressions. 1 2 3 4
47. Is too silly or laughs inappropriately. 1 2 3 4
48. Has a sense of humor, understands jokes. 1 2 3 4
49. Does extremely well at a few tasks, but does not do as well at most other tasks. 1 2 3 4
50. Has repetitive, odd behaviors such as hand flapping or rocking. 1 2 3 4
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52. Knows when he or she is talking too loud or making too much noise. 1 2 3 4
53. Talks to people with an unusual tone of voice (for example, talks like a robot or like he or she is giving a lecture). 1 2 3 4
54. Seems to react to people as if they are objects. 1 2 3 4
55. Knows when he or she is too close to someone or is invading someone's space. 1 2 3 4
56. Walks in between two people who are talking. 1 2 3 4
57. Gets teased a lot. 1 2 3 4
58. Concentrates too much on parts of things rather than seeing the whole picture. For example, if asked to describe what happened in a story, he or she may talk only about the kind of clothes the characters were wearing. 1 2 3 4
59. Is overly suspicious. 1 2 3 4
60. Is emotionally distant, doesn't show his or her feelings. 1 2 3 4
61. Is inflexible, has a hard time changing his or her mind. 1 2 3 4
62. Gives unusual or illogical reasons for doing things. 1 2 3 4
63. Touches others in an unusual way (for example, he or she may touch someone just to make contact and then walk away without saying anything). 1 2 3 4
64. Is too tense in social settings. 1 2 3 4
65. Stares or gazes off into space. 1 2 3 4

D4 **NICHQ Vanderbilt Assessment Scale—TEACHER Informant**

Teacher's Name: Mr. Daley Class Time: 1:10-2:28 Class Name/Period: Math

Today's Date: 10/27 Child's Name: Zoe Bitner Grade Level: 6

Directions: **Each rating should be considered in the context of what is appropriate for the age of the child you are rating and should reflect that child's behavior since the beginning of the school year. Please indicate the number of weeks or months you have been able to evaluate the behaviors:** 2 months

Is this evaluation based on a time when the child ☐ **was on medication** ☐ **was not on medication** ☑ **not sure?**

Symptoms	Never	Occasionally	Often	Very Often
1. Fails to give attention to details or makes careless mistakes in schoolwork	0	1	2	3
2. Has difficulty sustaining attention to tasks or activities	0	1	2	3
3. Does not seem to listen when spoken to directly	0	1	2	3
4. Does not follow through on instructions and fails to finish schoolwork (not due to oppositional behavior or failure to understand)	0	1	2	3
5. Has difficulty organizing tasks and activities	0	1	2	3
6. Avoids, dislikes, or is reluctant to engage in tasks that require sustained mental effort	0	1	2	3
7. Loses things necessary for tasks or activities (school assignments, pencils, or books)	0	1	2	3
8. Is easily distracted by extraneous stimuli	0	1	2	3
9. Is forgetful in daily activities	0	1	2	3
10. Fidgets with hands or feet or squirms in seat	0	1	2	3
11. Leaves seat in classroom or in other situations in which remaining seated is expected	0	1	2	3
12. Runs about or climbs excessively in situations in which remaining seated is expected	0	1	2	3
13. Has difficulty playing or engaging in leisure activities quietly	0	1	2	3
14. Is "on the go" or often acts as if "driven by a motor"	0	1	2	3
15. Talks excessively	0	1	2	3
16. Blurts out answers before questions have been completed	0	1	2	3
17. Has difficulty waiting in line	0	1	2	3
18. Interrupts or intrudes on others (eg, butts into conversations/games)	0	1	2	3
19. Loses temper	0	1	2	3
20. Actively defies or refuses to comply with adult's requests or rules	0	1	2	3
21. Is angry or resentful	0	1	2	3
22. Is spiteful and vindictive	0	1	2	3
23. Bullies, threatens, or intimidates others	0	1	2	3
24. Initiates physical fights	0	1	2	3
25. Lies to obtain goods for favors or to avoid obligations (eg, "cons" others)	0	1	2	3
26. Is physically cruel to people	0	1	2	3
27. Has stolen items of nontrivial value	0	1	2	3
28. Deliberately destroys others' property	0	1	2	3
29. Is fearful, anxious, or worried	0	1	2	3
30. Is self-conscious or easily embarrassed	0	1	2	3
31. Is afraid to try new things for fear of making mistakes	0	1	2	3

The recommendations in this publication do not indicate an exclusive course of treatment or serve as a standard of medical care. Variations, taking into account individual circumstances, may be appropriate.

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Adapted from the Vanderbilt Rating Scales developed by Mark L. Wolraich, MD.

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HE0351

D4 NICHQ Vanderbilt Assessment Scale—TEACHER Informant, continued

Teacher's Name: Daly Class Time: 1:10-2:28 Class Name/Period: Math

Today's Date: 10/27 Child's Name: Zoe Bitner Grade Level: 6

Symptoms (continued)	Never	Occasionally	Often	Very Often
32. Feels worthless or inferior	0	1	2	3
33. Blames self for problems; feels guilty	0	1	2	3
34. Feels lonely, unwanted, or unloved; complains that "no one loves him or her"	0	1	2	3
35. Is sad, unhappy, or depressed	0	1	2	3

Performance *Academic Performance*	Excellent	Above Average	Average	Somewhat of a Problem	Problematic
36. Reading	1	2	3	4	5
37. Mathematics	1	2	3	4	5
38. Written expression	1	2	3	4	5

Classroom Behavioral Performance	Excellent	Above Average	Average	Somewhat of a Problem	Problematic
39. Relationship with peers	1	2	3	4	5
40. Following directions	1	2	3	4	5
41. Disrupting class	1	2	3	4	5
42. Assignment completion	1	2	3	4	5
43. Organizational skills	1	2	3	4	5

Comments:

Please return this form to: _____

Mailing address: _____

_____

Fax number: _____

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Total number of questions scored 2 or 3 in questions 1–9: _____

Total number of questions scored 2 or 3 in questions 10–18: _____

Total Symptom Score for questions 1–18: _____

Total number of questions scored 2 or 3 in questions 19–28: _____

Total number of questions scored 2 or 3 in questions 29–35: _____

Total number of questions scored 4 or 5 in questions 36–43: _____

Average Performance Score: _____

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D4 **NICHQ Vanderbilt Assessment Scale—TEACHER Informant**

Teacher's Name: Weber Class Time: _____ Class Name/Period: SS / ELA

Today's Date: _____ Child's Name: Zoe Bitner Grade Level: 6

Directions: **Each rating should be considered in the context of what is appropriate for the age of the child you are rating and should reflect that child's behavior since the beginning of the school year. Please indicate the number of weeks or months you have been able to evaluate the behaviors:** 2 mths.

Is this evaluation based on a time when the child ☐ **was on medication** ☐ **was not on medication** ☑ **not sure?**

Symptoms	Never	Occasionally	Often	Very Often
1. Fails to give attention to details or makes careless mistakes in schoolwork	0	1	2	3
2. Has difficulty sustaining attention to tasks or activities	0	1	2	3
3. Does not seem to listen when spoken to directly	0	1	2	3
4. Does not follow through on instructions and fails to finish schoolwork (not due to oppositional behavior or failure to understand)	0	1	2	3
5. Has difficulty organizing tasks and activities	0	1	2	3
6. Avoids, dislikes, or is reluctant to engage in tasks that require sustained mental effort	0	1	2	3
7. Loses things necessary for tasks or activities (school assignments, pencils, or books)	0	1	2	3
8. Is easily distracted by extraneous stimuli	0	1	2	3
9. Is forgetful in daily activities	0	1	2	3
10. Fidgets with hands or feet or squirms in seat	0	1	2	3
11. Leaves seat in classroom or in other situations in which remaining seated is expected	0	1	2	3
12. Runs about or climbs excessively in situations in which remaining seated is expected	0	1	2	3
13. Has difficulty playing or engaging in leisure activities quietly	0	1	2	3
14. Is "on the go" or often acts as if "driven by a motor"	0	1	2	3
15. Talks excessively	0	1	2	3
16. Blurts out answers before questions have been completed	0	1	2	3
17. Has difficulty waiting in line	0	1	2	3
18. Interrupts or intrudes on others (eg, butts into conversations/games)	0	1	2	3
19. Loses temper	0	1	2	3
20. Actively defies or refuses to comply with adult's requests or rules	0	1	2	3
21. Is angry or resentful	0	1	2	3
22. Is spiteful and vindictive	0	1	2	3
23. Bullies, threatens, or intimidates others	0	1	2	3
24. Initiates physical fights	0	1	2	3
25. Lies to obtain goods for favors or to avoid obligations (eg, "cons" others)	0	1	2	3
26. Is physically cruel to people	0	1	2	3
27. Has stolen items of nontrivial value	0	1	2	3
28. Deliberately destroys others' property	0	1	2	3
29. Is fearful, anxious, or worried	0	1	2	3
30. Is self-conscious or easily embarrassed	0	1	2	3
31. Is afraid to try new things for fear of making mistakes	0	1	2	3

The recommendations in this publication do not indicate an exclusive course of treatment or serve as a standard of medical care. Variations, taking into account individual circumstances, may be appropriate.

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D4 NICHQ Vanderbilt Assessment Scale—TEACHER Informant, continued

Teacher's Name: _____ Class Time: _____ Class Name/Period: _____

Today's Date: _____ Child's Name: _____ Grade Level: _____

Symptoms (continued)	Never	Occasionally	Often	Very Often
32. Feels worthless or inferior	0	1	2	3
33. Blames self for problems; feels guilty	0	1	2	3
34. Feels lonely, unwanted, or unloved; complains that "no one loves him or her"	0	1	2	3
35. Is sad, unhappy, or depressed	0	1	2	3

Performance *Academic Performance*	Excellent	Above Average	Average	Somewhat of a Problem	Problematic
36. Reading	1	2	3	4	5
37. Mathematics	1	2	3	4	5
38. Written expression	1	2	3	4	5

Classroom Behavioral Performance	Excellent	Above Average	Average	Somewhat of a Problem	Problematic
39. Relationship with peers	1	2	3	4	5
40. Following directions	1	2	3	4	5
41. Disrupting class	1	2	3	4	5
42. Assignment completion	1	2	3	4	5
43. Organizational skills	1	2	3	4	5

Comments:

Please return this form to: _____

Mailing address: _____

_____

Fax number: _____

For Office Use Only

Total number of questions scored 2 or 3 in questions 1–9: _____

Total number of questions scored 2 or 3 in questions 10–18: _____

Total Symptom Score for questions 1–18: _____

Total number of questions scored 2 or 3 in questions 19–28: _____

Total number of questions scored 2 or 3 in questions 29–35: _____

Total number of questions scored 4 or 5 in questions 36–43: _____

Average Performance Score: _____

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11-20/rev0303

NICHQ Vanderbilt Assessment Scale—PARENT Informant

Today's Date: 11-3-26 Child's Name: Zoe Bitner Date of Birth: 8-28-14

Parent's Name: Sara Walker Parent's Phone Number: 412-722-8589

Directions: Each rating should be considered in the context of what is appropriate for the age of your child. When completing this form, please think about your child's behaviors in the past 6 months.

Is this evaluation based on a time when the child ☐ **was on medication** ☑ **was not on medication** ☐ **not sure?**

Symptoms	Never	Occasionally	Often	Very Often
1. Does not pay attention to details or makes careless mistakes with, for example, homework	0	1	2	(3)
2. Has difficulty keeping attention to what needs to be done	0	1	2	(3)
3. Does not seem to listen when spoken to directly	0	1	(2)	3
4. Does not follow through when given directions and fails to finish activities (not due to refusal or failure to understand)	0	1	2	(3)
5. Has difficulty organizing tasks and activities	0	1	2	(3)
6. Avoids, dislikes, or does not want to start tasks that require ongoing mental effort	0	1	2	(3)
7. Loses things necessary for tasks or activities (toys, assignments, pencils, or books)	0	1	2	(3)
8. Is easily distracted by noises or other stimuli	0	1	2	(3)
9. Is forgetful in daily activities	0	1	2	(3)
10. Fidgets with hands or feet or squirms in seat	0	1	2	(3)
11. Leaves seat when remaining seated is expected	0	(1)	2	3
12. Runs about or climbs too much when remaining seated is expected	(0)	1	2	3
13. Has difficulty playing or beginning quiet play activities	0	(1)	2	3
14. Is "on the go" or often acts as if "driven by a motor"	0	1	2	(3)
15. Talks too much	0	1	2	(3)
16. Blurts out answers before questions have been completed	0	1	2	(3)
17. Has difficulty waiting his or her turn	0	1	2	(3)
18. Interrupts or intrudes in on others' conversations and/or activities	0	1	2	(3)
19. Argues with adults	0	1	(2)	3
20. Loses temper	0	1	2	(3)
21. Actively defies or refuses to go along with adults' requests or rules	0	1	(2)	3
22. Deliberately annoys people	0	1	2	(3)
23. Blames others for his or her mistakes or misbehaviors	0	1	(2)	3
24. Is touchy or easily annoyed by others	0	1	2	(3)
25. Is angry or resentful	0	1	(2)	3
26. Is spiteful and wants to get even	0	1	(2)	3
27. Bullies, threatens, or intimidates others	0	(1)	2	3
28. Starts physical fights	(0)	1	2	3
29. Lies to get out of trouble or to avoid obligations (ie, "cons" others)	(0)	1	2	3
30. Is truant from school (skips school) without permission	(0)	1	2	3
31. Is physically cruel to people	(0)	1	2	3
32. Has stolen things that have value	(0)	1	2	3

The information contained in this publication should not be used as a substitute for the medical care and advice of your pediatrician. There may be variations in treatment that your pediatrician may recommend based on individual facts and circumstances.

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NICHQ Vanderbilt Assessment Scale—PARENT Informant

Today's Date: _____ Child's Name: _____ Date of Birth: _____

Parent's Name: _____ Parent's Phone Number: _____

Symptoms (continued)	Never	Occasionally	Often	Very Often
33. Deliberately destroys others' property	0	1	2	3
34. Has used a weapon that can cause serious harm (bat, knife, brick, gun)	0	1	2	3
35. Is physically cruel to animals	0	1	2	3
36. Has deliberately set fires to cause damage	0	1	2	3
37. Has broken into someone else's home, business, or car	0	1	2	3
38. Has stayed out at night without permission	0	1	2	3
39. Has run away from home overnight	0	1	2	3
40. Has forced someone into sexual activity	0	1	2	3
41. Is fearful, anxious, or worried	0	1	2	3
42. Is afraid to try new things for fear of making mistakes	0	1	2	3
43. Feels worthless or inferior	0	1	2	3
44. Blames self for problems, feels guilty	0	1	2	3
45. Feels lonely, unwanted, or unloved; complains that "no one loves him or her"	0	1	2	3
46. Is sad, unhappy, or depressed	0	1	2	3
47. Is self-conscious or easily embarrassed	0	1	2	3

Performance	Excellent	Above Average	Average	Somewhat of a Problem	Problematic
48. Overall school performance	1	2	3	4	5
49. Reading	1	2	3	4	5
50. Writing	1	2	3	4	5
51. Mathematics	1	2	3	4	5
52. Relationship with parents	1	2	3	4	5
53. Relationship with siblings	1	2	3	4	5
54. Relationship with peers	1	2	3	4	5
55. Participation in organized activities (eg, teams)	1	2	3	4	5

Comments:

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Total number of questions scored 2 or 3 in questions 1–9: 9

Total number of questions scored 2 or 3 in questions 10–18: 6

Total Symptom Score for questions 1–18: _____

Total number of questions scored 2 or 3 in questions 19–26: _____

Total number of questions scored 2 or 3 in questions 27–40: _____

Total number of questions scored 2 or 3 in questions 41–47: _____

Total number of questions scored 4 or 5 in questions 48–55: _____

Average Performance Score: _____

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Original: 10/1/86
First Revision: 3/1/90
Second Revision: 5/1/91
Third Revision: 5/1/93
Fourth Revision: 6/17/99
Fifth Revision: 10/04/07

CHILDREN'S YALE-BROWN OBSESSIVE COMPULSIVE SCALE (CY-BOCS)

DEVELOPED BY

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Investigators interested in using this rating scale should contact Lawrence Scahill, M.S.N., Ph.D., at the Yale Child Study Center, P.O. Box 207900, New Haven, CT 06520 or Wayne Goodman, M.D., at the National Institute of Mental Health, Bethesda, MD.

Scahill, L., Riddle, M.A., McSwiggin-Hardin, M., Ort, S.I., King, R.A., Goodman, W.K., Cicchetti, D. & Leckman, J.F. (1997). Children's Yale-Brown Obsessive Compulsive Scale: reliability and validity. *J Am Acad Child Adolesc Psychiatry*, 36(6):844-852.

GENERAL INSTRUCTIONS

Overview:

This scale is designed to rate the severity of obsessive and compulsive symptoms in children and adolescents, ages 6 to 17 years. It can be administered by a clinican or trained interviewer in a semi-structured fashion. In general, the ratings depend on the child's and parent's report; however, the final rating is based on the clinical judgement of the interviewer. Rate the characteristics of each item over the prior week up until, and including, the time of the interview. Scores should reflect the average of each item for the entire week, unless otherwise specified.

Informants:

Information should be obtained by interviewing the parent(s) (or guardian) and the child together. Sometimes, however, it may also be useful to interview the child or parent alone. Interviewing strategy may vary depending on the age and developmental level of the child or adolescent. All information should be combined to estimate the score for each item. Whenever the CY-BOCS is administered more than once to the same child, as in a medication trial, consistent reporting can be ensured by having the same informant(s) present at each rating session.

Definitions:

Before proceeding with the questions, define "obsessions" and "compulsions" for the child and primary caretaker as follows (sometimes, particularly with younger children, the interviewer may prefer using the terms "worries" and "habits"):

"OBSESSIONS: are thoughts, ideas, or pictures that keep coming into your mind even though you do not want them to. They may be unpleasant, silly or embarrassing."

"AN EXAMPLE OF AN OBSESSION IS: the repeated thought that germs or dirt are harming you or other people, or that something unpleasant might happen to you or someone in your family or someone special to you. These are thoughts that keep coming back, over and over again."

"COMPULSIONS: are things that you feel you have to do although you may know that they do not make sense. Sometimes you may try to stop from doing them but this might not be possible. You might feel worried or angry or frustrated until you have finished what you have to do."

"AN EXAMPLE OF A COMPULSION IS: the need to wash your hands over and over again even though they are not really dirty, or the need to count up to a certain number while you do certain things."

"Do you have any questions about what these words called obsessions and compulsions mean?"

Symptom Specificity and Continuity:

In some cases, it may be difficult to delineate obsessions and compulsions from other closely related symptoms such as phobias, anxious worries, depressive ruminations or complex tics. Separate assessment of these symptoms may be necessary. Although potentially difficult, the delineation of obsessions and compulsions from these closely related symptoms is an essential task of the interviewer. (A full discussion of how to make this determination is beyond the scope and purpose of this introduction.) Items marked with an asterix are items where this delineation may be especially troublesome.

Once the interviewer has decided whether or not a particular symptom will be included as an obsession or compulsion on the checklist, every effort should be made to maintain consistency in subsequent rating(s). In a treatment study with multiple ratings over time, it may be useful to review the initial Target Symptom Checklist (see below) at the beginning of subsequent ratings (prior severity scores should not be reviewed).

Procedure:

Symptom Checklist: After reviewing with the child and parent(s) the definitions of obsessions and compulsions, the interview should proceed with a detailed inquiry about the child's symptoms using the Compulsions Checklist and Obsessions Checklist as guides. It may not be necessary to ask about each and every item on the checklist, but each symptom area should be covered to ensure that symptoms are not missed. For most children and adolescents, it is usually easier to begin with compulsions (pages 9 and 10).

Target Symptom List: After the Compulsions Checklist is complete, list the four most severe compulsions on the Target Symptom List on page 10. Repeat this process, listing the most severe obsessions, on the Target Symptom List on page 5.

Severity Rating: After completing the Checklist and Target Symptom List for compulsions, inquire about the severity items: Time Spent, Distress, Resistance, Interference, and Degree of Control (questions 6 through 10 on pages 11 through 13). There are examples of probe questions for each item. Ratings for these items should reflect interviewer's best estimate from all available information from the past week, with special emphasis on the Target Symptoms. Repeat the above procedure for obsessions (Pages 4 through 8). Finally, inquire about and rate questions 11 through 19 on pages 14 and 18. Scores can be recorded on the scoring sheet on page 19. All ratings should be in whole integers.

Scoring:

All 19 items are rated, but only items 1-10 are used to determine the total score. The **total CY-BOCS score** is the sum of items 1-10; the **obsession and compulsion subtotals** are the sums of items 1-5 and 6-10, respectively. At this time, items 1A and 6A are not being used in the scoring.

Items 17 (global severity) and 18 (global improvement) are adapted from the Clinical Global Impression Scale (Guy, W., 1976) to provide measures of overall functional impairment associated with the presence of obsessive-compulsive symptoms.

Name _____ Date _____

CY-BOCS OBSESSIONS CHECKLIST

Check all items that apply (Item marked "*" may or not be OCD phenomena.)

Current	Past	Contamination Obsessions
___	___	Concern with dirt, germs, certain illnesses (e.g., AIDS)
___	___	Concerns or disgust with bodily waste or secretions (e.g., urine, feces, saliva)
___	___	Excessive concern with enviromental contaminants (e.g., asbestos, radiation, toxic waste)
___	___	Excessive concern with household items (e.g., cleaners, solvents)
___	___	Excessive concern about animals/insects
___	___	Excessively bothered by sticky substances or residues
___	___	Concerned will get ill because of contaminant
___	___	Concerned will get others ill by spreading contaminant (aggressive)
___	___	No concern with consequences of contamination other than how it might feel *
___	___	Other (Describe) _____

Aggressive Obsessions

___	___	Fear might harm self
___	___	Fear might harm others
___	___	Fear harm will come to self
X	___	Fear harm will come to others (may be because something child did or did not do)
___	___	Violent or horrific images
___	___	Fear of blurting out obscenities or insults
X	___	Fear of doing something else embarrassing *
___	___	Fear will act on unwanted impulses (e.g. to stab a family member)
___	___	Fear will steal things
___	___	Fear will be responsible for something else terrible happening (e.g. fire, burglary, flood)
___	___	Other (Describe) _____

family might die randomly LD Not my fault

do something random (maybe not obsessions)

Sexual Obsessions

[Are you having any sexual thoughts? If yes, are they routine or are they repetitive thoughts that you would rather not have or find disturbing? If yes, are they:]

___	___	Forbidden or perverse sexual thoughts, images, impulses
___	___	Content involves homosexuality *
___	___	Sexual behavior towards others (Aggressive)
___	___	Other (Describe) _____

Hoarding/Saving Obsessions

X	___	Fear of losing things
___	___	Other (Describe) _____

Won't be able to find

Magical Thoughts/Superstitous Obsessions

X	___	Lucky/unlucky numbers, colors, words
___	___	Other (Describe) _____

something may be bad or unlucky

(don't like pairs or disorganized #s

Current	Past	Somatic Obsessions
___	___	Excessive concern with illness or disease *
___	___	Excessive concern with body part or aspect of appearance (e.g., dysmorphophobia) *
___	___	Other (Describe) _____

		Religious Obsessions (Scrupulosity)
___	___	Excessive concern or fear of offending religious objects (God)
___	___	Excessive concern with right/wrong, morality
___	___	Other (Describe) _____

		Miscellaneous Obsessions
X	___	The need to know or remember
___	___	Fear of saying certin things
___	___	Fear of not saying just the right thing
___	___	Intrusive (non-violent) images
___	___	Intrusive sounds, words, music, or numbers
___	___	Other (Describe) _____

Will forget something very impotant

TARGET SYMPTOM LIST FOR OBSESSIONS

Obsessions (Describe, listing by order of severity, with #1 being the most severe, #2 the second most severe, etc.):

1. family friends night
2. disorganized
3. forgetting
4. loss

QUESTIONS ON OBSESSIONS (ITEMS 1-5) "I AM NOW GOING TO ASK YOU QUESTIONS ABOUT THE THOUGHTS YOU CANNOT STOP THINKING ABOUT." (Review for the informant(s) the Target Symptoms and refer to them while asking questions 1-5).

1. Time Occupied by Obsessive Thoughts

• How much time do you spend thinking about these things?
(When obsessions occur as brief, intermittent intrusions, it may be impossible to assess time occupied by them in terms of total hours. In such cases, estimate time by determining how frequently they occur. Consider both the number of times the intrusions occur and how many hours of the day are affected).

• How frequently do these thoughts occur?
[Exclude ruminations and preoccupations which, unlike obsessions, are ego-syntonic and rational (but exaggerated).]

0 - NONE few

1 - MILD less than 1 hr/day or occasional intrusion few mild (< 1 hr)

2 - MODERATE 1 to 3 hrs/day or frequent intrusion

3 - SEVERE greater than 3 and up to 8 hrs/day or very frequent intrusion

4 - EXTREME greater than 8 hrs/day or near constant intrusion

1B. Obsession-free Interval (not included in total score)

• On average, what is the longest amount of time per day that you are not bothered by obsessivethoughts?

0 - NONE

1 - MILD long symptom free intervals, more than 8 consecutive hrs/day symptom-free

2 - MODERATE moderately long symptom-free intervals, more than 3 and up to 8 hrs/day

3 - SEVERE brief symptom-free intervals, from 1 to 3 consecutive hrs/day symptom-free

4 - EXTREME less than 1 consecutive hr/day symptom free

2. Interference due to Obsessive Thoughts

• How much do these thoughts get in the way of school or doing things with friends?

• Is there anything that you don't do because of them?

(If currently not in school determine how much performance would be affected if patient were in school.)

0 - NONE

1 - MILD slight interference with social or school activities, overall performance not impaired

2 - MODERATE definite interference with social or school performance, but still manageable

3 - SEVERE causes substantial impairment in social or school performance

4 - EXTREME incapacitating

3. Distress Associated with Obsesssive Thoughts

• How much do these thoughts bother or upset you?

(Only rate anxiety/frustration that seems triggered by obsessions, not generalized anxiety or anxiety associated with other symptoms.)

0 - NONE

1 - MILD infrequent, and not too disturbing

2 - MODERATE frequent, and disturbing, but still manageable

3 - SEVERE very frequent, and very disturbing

4 - EXTREME near constant, and disabling distress/frustration

4. Resistance Against Obsessions

• How hard do you try to stop the thoughts or ignore them?

(Only rate effort made to resist, not success or failure in actually controlling the obsessions. How much patient resists the obsessions may or may not correlate with their ability to control them. Note that this item does not directly measure the severity of the intrusive thoughts; rather it rates a manifestation of health, i.e., the effort the patient makes to counteract the obsessions. Thus, the more the patient tries to resist, the less impaired is this aspect of his functioning. If the obsessions are minimal, the patient may not feel the need to resist them. In such cases, a rating of "0" should be given.)

0 – NONE makes an effort to always resist, or symptoms so minimal doesn't need to actively resist.

1 - MILD tries to resist most of the time

2 - MODERATE makes some effort to resist

3 - SEVERE yields to all obsessions without attempting to control them, but does so with some reluctance

4 – EXTREME completely and willingly yields to all obsessions

5. Degree of Control Over Obsessive Thoughts

• When you try to fight the thoughts, can you beat them?

• How much control do you have over the thoughts?

(In contrast to the preceding item on resistance, the ability of the patient to control his obsessions is more closely related to the severity of the intrusive thoughts.

0 - COMPLETE CONTROL

1 - MUCH CONTROL usually able to stop or divert obsessions with some effort and concentration.

2 - MODERATE CONTROL sometimes able to stop or divert obsessions

3 - LITTLE CONTROL rarely successful in stopping obsessions, can only divert attention with difficulty

4 - NO CONTROL experienced as completely involuntary, rarely able to even momentarily divert thinking

Name _____ Date _____

CY-BOCS COMPULSIONS CHECKLIST

Check all items that apply (Item marked "*" may or not be OCD phenomena.)

Current	Past	
		Washing/Cleaning Compulsions
___	___	Excessive or ritualized handwashing
___	___	Excessive or ritualized showering, bathing, toothbrushing, grooming, or toilet routine
___	___	Excessive cleaning of items; such as personal clothes or important objects
___	___	Other measures to prevent or remove contact with contaminants
X	___	Other (Describe) Cleans car mats
		Checking Compulsions
X	___	Checking locks, toys, school books/items, etc. – Mom room Bro
___	___	Checking associated with getting washed, dressed, or undressed. bed each
___	___	Checking that did not/will not harm others
___	___	Checking that did not/will not harm self
X	___	Checking that nothing terrible did/will happen – D Checking
X	___	Checking that did not make mistake in here Really?
___	___	Checking tied to somatic obsessions
___	___	Other (Describe) _____
		Repeating Rituals
___	___	Rereading, erasing, or rewriting
___	___	Need to repeat routine activities (e.g. in/out doors, up/down from chair)
___	___	Other (Describe) _____
		Counting Compulsions
X	___	Objects, certain numbers, words, etc. – D Just county Describe: _____
		Ordering/Arranging
X	___	Need for symmetry/evening up (e.g., lining items up a certain way or arranging personal items in specific patterns)
___	___	Other (Describe) _____
		Hoarding/Saving Compulsion
		[distinguish from hobbies and concern with objects of monetary or sentimental value]
X	___	Difficulty throwing things away, saving bits of paper, string, etc. D
___	___	Other (Describe) _____
		Excessive Games/Superstitious Behaviors Package of toys; car wheels chris
		[distinguish from age appropriate magical games]
		(e.g., array of behavior, such as stepping over certain spots on a floor, touching an object/self certain number of times as a routine game to avoid something bad from happening.)
___	___	Other (Describe) _____

Current Past Rituals Involving Other Persons

The need to involve another person (usually a parent) in ritual (e.g., asking a parent to repeatedly answer the same question, making mother perform certain meal time-rituals involving specific utensils).*

____ ____ Other (Describe) _____

Miscellaneous Compulsions

____ ____ Mental rituals (other than checking/counting)
____ ____ Need to tell, ask, or confess
____ ____ Measures (not checking) to prevent harm to self__; harm to others___; terrible consequences ___
____ ____ Ritualized eating behaviors *
____ ____ Excessive list making *
____ ____ Need to touch, tap, rub *
____ ____ Need to do things (e.g., touch or arrange) until it feels just right) *
____ ____ Rituals involving blinking or staring *
____ ____ Trichotillomanis (hair-pulling) *
____ ____ Other self-damaging or self-mutilating behaviors *
____ ____ Other (Describe) _____

TARGET SYMPTOM LIST FOR COMPULSIONS

Compulsions (Describe, listing by order of severity, with #1 being the most severe, #2 second most severe, etc.):

1. Clearing under nails
2. ~~counting/organizing~~; throw away
3. Counts of Any
4. back Pack

QUESTIONS ON COMPULSIONS (ITEMS 6-10) "I AM NOW GOING TO ASK YOU QUESTIONS ABOUT THE HABITS YOU CAN'T STOP." (Review for the informant(s) the Target Symptoms and refer to them while asking questions 6-10).

6A. Time Spent Performing Compulsive Behaviors

- How much time do you spend doing these things?
- How much longer than most people does it take to complete your usual daily activities because of the habits?

  (When compulsions occur as brief, intermittent behaviors, it may be impossible to assess time spent performing them in terms of total hours. In such cases, estimate time by determining how frequently they are performed. Consider both the number of times compulsions are performed and how many hours of the day are affected.)
- How often do you do these habits?

  [In most cases compulsions are observable behaviors (e.g., handwashing), but there are instances in which compulsions are not observable (e.g., silent checking).]

30 minutes total

0 - NONE	
1 - MILD	(spends less than 1 hr/day performing compulsions), or occasional performance of compulsive behaviors
2 - MODERATE	(spends from 1 to 3 hrs/day performing compulsions), or frequent performance of compulsive behaviors
3 - SEVERE	(spends more than 3 and up to 8 hrs/day performing compulsions), or very frequent performance of compulsive behaviors
4 - EXTREME	(spends more than 8 hrs/day performing compulsions), or near constant performance of compulsive behaviors (too numerous to count).

6B. Compulsion-free Interval

- How long can you go without performing compulsive behavior?

  [If necessary ask: What is the longest block of time in which (your habits) compulsions are absent?]

0 - NO SYMPTOMS	
1 - MILD	long symptom-free interval, more than 8 consecutive hrs/day symptom-free
2 - MODERATE	moderately long symptom-free interval, more than 3 and up to 8 consecutive hrs/day symptom-free.
3 - SEVERE	short symptom-free interval, from 1 to 3 consecutive hrs/day symptom free
4 - EXTREME	less than 1 consecutive hr/day symptom-free

7. Interference due to Compulsive Behaviors

- How much do these habits get in the way of school or doing things with friends?
- Is there anything you don't do because of them?

(If currently not in school, determine how much performance would be affected if patient were in school.)

0 - NONE	
1 - MILD	slight, interference with social or school activities, but overall performance not impaired
2 – MODERATE	definite interference with social or school performance, but still manageable
3 - SEVERE	causes substantial impairment in social or school performance
4 - EXTREME	incapacitating

8. Distress Associated with Compulsive Behavior

- How would you feel if prevented from carrying out your habits?
- How upset would you become?

(Rate degree of distress/frustration patient would experience if performance of the compulsion were suddenly interrupted without reassurance offered. In most, but not all cases, performing compulsions reduces anxiety /frustration.)

- How upset do you get while carrying out your habits until you are satisfied?

0 - NONE	
1 - MILD	only slightly anxious/frustrated if compulsions prevented, or only slight anxiety/frustration during performance of compulsions.
2 - MODERATE	reports that anxiety/frustration would mount but remain manageable if compulsions prevented. Anxiety/frustration increases but remains manageable during performance of compulsions.
3 - SEVERE	prominent and very disturbing increase in anxiety/frustration if compulsions interrupted. Prominent and very disturbing increase in anxiety /frustration during performance of compulsions.
4 - EXTREME	incapacitating anxiety/frustration from any intervention aimed at modifying activity. Incapacitating anxiety/frustration develops during performance of compulsions.

9. Resistance Against Compulsions

• How much do you try to fight the habits?

(Only rate effort made to resist, not success or failure in actually controlling the compulsions. How much the patient resists the compulsions may or may not correlate with his ability to control them. Note that this item does not directly measure the severity of the compulsions; rather it rates a manifestation of health, i.e., the effort the patient makes to counteract the compulsions. Thus, the more the patient tries to resist, the less impaired is this aspect of their functioning. If the compulsions are minimal, the patient may not feel the need to resist them. In such cases, a rating of "0" should be given.)

0 - NONE	Makes an effort to always resist, or symptoms so minimal doesn't need to actively resist.
1 - MILD	Tries to resist most of the time.
2 - MODERATE	Makes some effort to resist
3 - SEVERE	Yields to almost all compulsions without attempting to control them, but does so with some reluctance.
4 - EXTREME	completely and willingly yields to all compulsions

10. Degree of Control over Compulsive Behavior

• How strong is the feeling that you have to carry out the habit(s)?

• When you try to fight them what happens?

(For the advanced child ask:)

• How much control do you have over the habits?

(In contrast to the preceding item on resistance, the ability of the patient to control his compulsions is closely related to the severity of the compulsions.)

0 - COMPLETE CONTROL	
1 - MUCH CONTROL	experiences pressure to perform the behavior, but usually able to exercise voluntary control over it
2 - MODERATE CONTROL	moderate control, strong pressure to perform behavior, can control it only with difficulty
3 - LITTLE CONTROL	little control, very strong drive to perform behavior, must be carried to completion, can only delay with difficulty
4 - NO CONTROL	no control, drive to perform behavior experienced as completely involuntary and overpowering, rarely able to delay activity (even momentarily)

CHILDREN'S YALE-BROWN OBSESSIVE COMPUSLIVE SCALE

CYBOCS TOTAL (add items 1-10) _____

Patient Name _____ Date _____ Patient ID _____

Rater _____

1. TIME SPENT ON OBSESIONS	0	1	2	3	4
1b. OBSESSION-FREE INTERVAL (do not add to subtotal or total score)	No Symptoms 0	Long 1	Moderately Long 2	Short 3	Extremely Short 4
2. INTERFERENCE FROM OBSESSIONS	0	1	2	3	4
3. DISTRESS OF OBSESSIONS	0	1	2	3	4
4. RESISTANCE	Always resists 0	1	2	3	Completely yields 4
5. CONTROL OVER OBSESSIONS	Complete control 0	Much control 1	Moderate control 2	Little control 3	No control 4

OBSESSION SUBTOTAL (add items 1-5) [ ]

6. TIME SPENT ON COMPULSIONS	0	1	2	3	4
6b. COMPULSION-FREE INTERVAL (do not add to subtotal or total score)	No Symptoms 0	Long 1	Moderately Long 2	Short 3	Extremely Short 4
7. INTERFERENCE FROM COMPULSION	0	1	2	3	4
8. DISTRESS FROM COMPULSIONS	0	1	2	3	4
9. RESISTANCE	Always resists 0	1	2	3	Completely yields 4
10. CONTROL OVER COMPULSIONS	Complete control 0	Much control 1	Moderate control 2	Little control 3	No control 4

COMPULSION SUBTOTAL (add items 6-10) [ ]

~~19. RELIABILITY EXCELLENT = 0 GOOD = 1 FAIR = 2 POOR = 3~~

EXCELLENT = no reason to suspect data unreliable; GOOD= factor(s) that may adversely affect reliability; FAIR= factor(s) that definitely reduce reliability; POOR= very low reliability.

D-KEFS Trail Making Test: Summary of Scores

Primary Measures: Completion Times

straight lines, one at a time

Condition 1: Visual Scanning	Condition 2: Number Sequencing	Condition 3: Letter Sequencing	Condition 4: Number–Letter Switching	Condition 5: Motor Speed
31 → 8	55 → 8	44 → 9	117 → 8	34 → 11
Raw Score → Scaled Score	Raw Score → Scaled Score	Raw Score → Scaled Score	Raw Score → Scaled Score	Raw Score → Scaled Score

Primary Combined Measure: Completion Times

	Number Sequencing		Letter Sequencing			Composite Scaled Score
Combined Number Sequencing + Letter Sequencing	8	+	9	=	17 →	
	Scaled Score		Scaled Score		Sum of Scaled Scores	

Primary Contrast Measures: Completion Times

	Switching: Scaled Score		Scaled Score		Scaled Score Difference	Contrast Scaled Score*
			Visual Scanning			
Number–Letter Switching vs. Visual Scanning*		–		=	→	
			Number Sequencing			
Number–Letter Switching vs. Number Sequencing*		–		=	→	
			Letter Sequencing			
Number–Letter Switching vs. Letter Sequencing*		–		=	→	
			Number Sequencing + Letter Sequencing			
Number–Letter Switching vs. Combined Number Sequencing + Letter Sequencing*		–	Composite	=	→	
			Motor Speed			
Number–Letter Switching vs. Motor Speed*		–		=	→	

* A low or high contrast scaled score may reflect different cognitive problems; see examiner's manual.

Optional Measures: Error Analysis

	Condition 1: Visual Scanning	Condition 2: Number Sequencing	Condition 3: Letter Sequencing	Condition 4: Number–Letter Switching	Condition 5: Motor Speed
Omission Errors	Raw Score: 0; Cumulative Percentile Rank:				
Commission Errors	Raw Score: 0; Cumulative Percentile Rank:				
Sequencing Errors		Raw Score: 0; Cumulative Percentile Rank:	Raw Score: 0; Cumulative Percentile Rank:	Raw Score: 3; Cumulative Percentile Rank:	
Set-Loss Errors		Raw Score: 2; Cumulative Percentile Rank:	Raw Score: 0; Cumulative Percentile Rank:	+ Raw Score: 0; Cumulative Percentile Rank:	
Time-Discontinue Errors		Raw Score: 0; Cumulative Percentile Rank:	Raw Score: 3; Cumulative Percentile Rank:	+ Raw Score: 0; Cumulative Percentile Rank:	Raw Score: ; Cumulative Percentile Rank:

Condition 4: All Error Types — Total Raw Score: ; Scaled Score:

Note: Cumulative percentile ranks for the D-KEFS were scaled to reflect the percentage of the normative sample that obtained raw scores equal to or worse than the raw score obtained by the examinee.

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Delis–Kaplan Executive Function System

Dean C. Delis Edith Kaplan Joel H. Kramer

D-KEFS Trail Making Test: Record Form

Name: _____ ID: _____ Examiner: _____

Sex: ❑ F ❑ M Handedness: ❑ R ❑ L ❑ Ambidextrous

Highest Level of Education (years): _____

Current Grade (if applicable): _____

School (if applicable): _____

	Year	Month	Day
Date Tested			
Date of Birth			
Age at Testing			

Referral Source/Reason for Referral/Presenting Complaints: _____

Attitude Toward Testing: _____

Affect and Mood: _____

Unusual Behaviors and Comments: _____

Physical Appearance: _____

Visual/Auditory/Motor Problems: _____

Language Background: _____

Diagnostic History: _____

Current Medications: _____

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